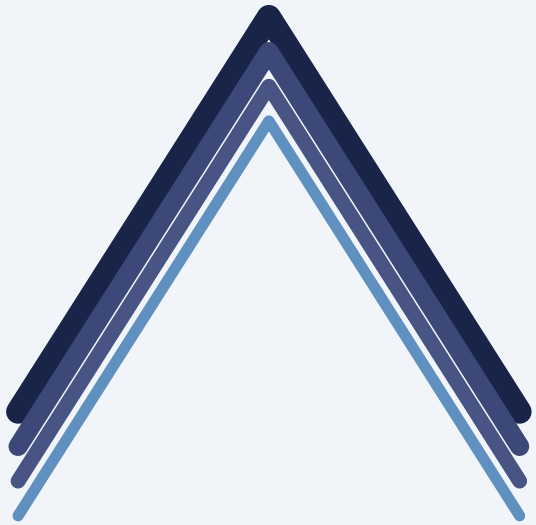
CONSULT FOR AFRICA
Build. Transform. Grow.

PREPARED FOR GRANDVILLE MEDICAL GROUP

Marketing built around how a hospital actually earns

Most agencies sell hospitals impressions. We have run hospitals.



PREPARED FOR	PREPARED BY	OFFICES	DATE
Grandville Medical Group	Consult for Africa	Lagos and Abuja	October 2026

Private and confidential. Prepared for a first conversation and not for onward circulation.

What we can see from outside, and what we cannot.

Thirteen years from a single plastic surgery practice to three centres, a trauma and critical care investment, and a service list that now runs from rhinoplasty to resuscitation. That is an operating story before it is a marketing story, and it is why we asked for the hour.

- **What is public.** Lekki, Surulere and Maitama. Aesthetic and reconstructive surgery alongside trauma and intensive care, laser, imaging, laboratory, dermatology and obstetrics. A med spa sub brand. Airport pickup and an aftercare home. A stated intention to reach all six geopolitical zones.
- **What is not.** Your margin by service line. Theatre, chair and bed utilisation by site. How many enquiries arrive in a month and how many become appointments. Where your patients travel from. Which centre is carrying which.
- **Why that matters.** Every recommendation worth making depends on those five things. A firm that has not asked for them is not selling you advice, it is selling you activity.

We would rather spend this hour asking than presenting. These slides exist so you know who is asking.

WHO IS ASKING

A healthcare management consulting firm that also runs a marketing practice.

Consult for Africa works from Lagos and Abuja across seven service lines: clinical governance, strategy and growth, fractional leadership, digital health, health systems strengthening, diaspora expertise and healthcare marketing. The work arrives as organisational audits, turnaround and growth mandates, board governance, venture structuring, and build and operate mandates where we stand a facility up and then manage it.

> **Most agencies sell hospitals impressions. We have run hospitals, so we sell admissions, referrals and the service lines that carry margin.**

- **We read a profit and loss.** The conversation starts with your service mix, your tariff and your utilisation, not with a follower count.
- **We will say when marketing is not the answer.** Sometimes the constraint is a clinic timetable, an unanswered telephone, or a referring doctor nobody has called in a year.
- **Strategy, creative, technology and analytics sit in one place.** Coordinating four vendors is a management job you should not have to hold.

Who would be on this, and what each of them owns.

Who	What they own on an engagement like this
Dr Debo Odulana	Founding Partner. Owns the diagnosis and stays on the engagement. Physician, with a masters in international health management from Imperial College Business School. Former Chief Executive of Cedarcrest Hospitals, Abuja. Former Chief Innovation and Strategy Officer at Evercare Hospital Lekki. Founded Doctoora, built it to twenty one states and sold it to Evercare.
Toluwanimi Olayeni	Marketing and communications lead. Owns the account day to day: the brief, the calendar, the clearance gate and the monthly report.
Tito Ipinmoye	Director of Product and Strategy. Owns execution wherever the work touches operations, data or the digital estate. Clinician and healthcare executive.

Nothing published in your name leaves this firm without a clinician checking the clinical claim and a named person clearing it. That gate is the most important thing we own.

You are running two marketing businesses, and they need opposite strategies.

An elective aesthetic procedure and an emergency admission are not the same product, sold to the same person, in the same way. They differ in who chooses, how long the decision takes, how far the patient will travel and what evidence closes it. A group that spans both usually markets one of them well and the other by accident.

The elective side	The acute and diagnostic side
The patient chooses, deliberately, over weeks or months	A family, a referring doctor or a payer chooses, in minutes
Won on authority, outcomes and visible proof	Won on proximity, an answered telephone and ambulance time
Catchment is national, and then diaspora	Catchment is the drive to the door
Price is researched and compared before contact	Price is discovered after the decision is made
Demand can be created	Demand is captured, or lost to whoever is nearer

A med spa sub brand suggests you have already felt this. The open question is whether the rest of the architecture follows it.

The founder's name is your most valuable asset and your largest concentration risk.

In aesthetics, the surgeon is the product. Nobody buys a rhinoplasty from an institution, and a reputation built over thirteen years is worth more than any campaign could buy. In trauma, intensive care, imaging and obstetrics that same name does very little work, and it cannot be in six zones at once.

- **Where the person stays primary.** Aesthetic and reconstructive surgery, the national and diaspora elective demand, and anything where a patient is choosing a pair of hands.
- **Where an institution has to carry it.** Emergency and critical care, diagnostics, obstetrics, the employer and payer relationships, and every site that will open without him standing in it.
- **Why this is urgent rather than interesting.** You cannot franchise a surgeon. If the six zone plan is real, the institutional brand has to be built before the expansion, not explained after it.

We have argued this both ways. For a surgeon whose craft is the product, we made the person primary. For a group that had to work without its founder, the institution. Which you are is a question about your plan, not your brand.

THE RULE WE BRING WITH US

We will not run a campaign into a service you cannot deliver.

The most common way a healthcare campaign fails is that it works. Enquiries arrive, the second call goes unanswered, the next appointment is three weeks out, and the budget has bought a worse reputation than you started with. So we ask about chairs, lists, beds and the telephone before we ask about reach.

- **What we ask for first.** Theatre and consulting room utilisation by site, bed occupancy, the share of enquiries answered on the first attempt, and the wait to the next available appointment.
- **What happens if the answer is uncomfortable.** We say so in writing, and we fix the front door before a naira goes to media.
- **What the rule is worth to you.** It is the difference between a campaign that fills a quiet list and one that lengthens a queue your competitor is happy to shorten.

> **Your capacity to serve demand is not our responsibility. Raising it before launch is.**

What marketing can move in a hospital group, and what it cannot.

Marketing can move this	Marketing cannot move this
Which service lines grow, and therefore your mix	Your clinical outcomes
Elective volume, and the self pay share of it	What a payer will agree to pay you
Referring doctor flow, by name and by volume	A consultant's availability on a Tuesday
The share of enquiries that become appointments	How a patient is treated at reception
Where your patients travel from	A theatre that is already full
Whether an employer or a payer knows you exist	A service you have not staffed

The left column is what we take accountability for. We will tell you plainly when a growth problem lives in the right column, because paying an agency to solve one of those is money burned.

You have half built a destination product and you are not selling it as one.

Airport pickup, an aftercare home and a recovery programme are the hard parts of inbound surgical care, and you already have them. What we cannot find is the packaged product: one price, one pathway, a named coordinator, and a written answer to the question every travelling patient actually asks, which is what happens if something goes wrong after I fly home.

- **The distinction most pitches miss.** For a patient in the United States the case is price. For a patient in the United Kingdom it is rarely price, it is rationing, age limits and waiting. Selling to the diaspora as one audience is the standard mistake.
- **What the group has that a clinic cannot claim.** Intensive care behind the theatre. The travelling patient's real fear is the complication that declares itself after the flight, and a credible answer to it is worth more than any discount.
- **What it still needs.** A pathway, a price, a coordinator, a complications promise, and relationships with the clinics abroad that see these patients first.

Eight capability areas, three ways to buy them.

Capability	What sits under it
Brand strategy and positioning	Architecture, value proposition, patient segmentation, naming, identity, guidelines and the governance that keeps them applied
Market research and insight	Epidemiological and competitive analysis, patient journey mapping, focus groups, digital listening
Campaign planning and management	Multichannel media planning, patient recruitment campaigns, advocacy programmes
Creative and content	Medical and consumer copywriting, design, video, and patient stories that have been medically vetted and consented
Digital and technology	Website and application build, search, social, email and messaging, booking and enquiry capture
Referrer and community engagement	Clinician referrer programmes, patient advocate networks, community management
Public relations and advertising	Press and health journalism, media training for spokespeople, print, outdoor and broadcast
Analytics and optimisation	Campaign dashboards, enquiry tracking, lead nurturing, testing and return on investment analysis

Retainer, project, or a retained strategic core with project work layered on top. The shape follows the work, and the next slides say which we would recommend here.

With your marketing team, not around it.

You have a marketing lead. The fastest way for an outside firm to become useless is to arrive as a replacement, because the person who knows your clinicians, your calendar and your consent rules is already on your payroll. Our default is to make that person better resourced and better armed.

Mode	What it means in practice
Augment, our default	Your lead keeps the brief, the relationships and the accountability. We bring capacity, specialist skills you do not need full time, and the instrumentation that makes their reporting defensible in front of your board.
Embed a fractional lead	A senior marketer inside your leadership rhythm for a set number of days a month, for when the gap is seniority rather than hands.
Take a defined scope end to end	One build, one service line launch or one campaign, with a start, a finish and a named deliverable.

The first two things we build on any account are the asset library and the measurement. Both of them make your own team faster after we have gone.

Seven standard shapes, because bespoke is slow and expensive for both of us.

We sell known shapes with known deliverables and known effort. Bespoke is available, by exception and at a premium, and anything outside a shape is quoted as a change in writing before it starts.

Shape	What you get	Duration
Brand foundation	Positioning, architecture, identity, guidelines and core messaging, applied to three surfaces	4 to 6 weeks
Digital front door	Website or landing estate, search foundations, enquiry and booking capture, analytics, and the how did you hear about us instrumentation	6 to 8 weeks
Service line launch	One service line positioned, creative, a campaign, referrer communications and a measurement frame	8 weeks
Referral engine	Referrer mapping, communications, a portal or maintained list, and the rhythm that keeps it alive	8 to 12 weeks
Always on marketing	Content calendar, social, email, community management and monthly reporting	Monthly
Growth retainer	Always on, plus paid media management, landing pages, testing and optimisation	Monthly
Fractional marketing lead	A senior marketer in your leadership rhythm for a set number of days a month	Monthly

Service levels, in writing, in the contract.

An agency that will not commit to response times in a contract is telling you something about its capacity. These are ours.

Commitment	Standard
Acknowledge a request	Same working day
Substantive response to a brief	Two working days
First draft of a standard deliverable	Five working days from an approved brief
Rounds of revision included	Two, after which it is a change of scope
Monthly report delivered	By the fifth working day of the following month
Campaign live after final approval	Three working days
A reputational issue	Four hours, in hours

We plan the team to seventy per cent of capacity. The slack is not waste, it is what makes the table on the left keepable in a month when something goes wrong.

What we will not do.

- **Take a commission on your media spend.** Media and third party production pass through at cost and you see the invoices. This is unusual in this market and it is the reason we can argue about a channel honestly.
- **Fund your media.** Campaign budget is yours and is in place before a campaign starts. An agency carrying a client's media money is an agency that one day stops working.
- **Claim an outcome we cannot evidence.** We report what is attributable, say plainly what is not, and will not put a number in front of you that we would not defend to your board.
- **Name you without written clearance.** Not in a case study, not in a pitch, not on a slide. Our proof is the method, and the method publishes without the client.
- **Market a service you cannot deliver.** The capacity rule, again, because it is the one most often broken.

What we actually do, shown without naming anybody.

We clear every named reference in writing, and most clients would rather we did not ask. So we will not show you logos. We will show you method, which is the thing you are buying.

The engagement	What the method found
A specialist facility whose growth plan rested on its revenue record	The revenue record was a nursing handover sheet. We rebuilt it from source before anybody was allowed to discuss growth.
A surgeon convinced his problem was his location	His problem was that the complex work he was best in the country at was not what any of his marketing described.
A campus whose business case rested on its floor area	The measured drawings were more than two hundred square metres away from the brief. Every figure downstream of it changed.
A health plan that wanted to count its providers	The number that mattered was coverage, not count. Counting harder would have answered the wrong question very precisely.
A ten case trace through a private surgical practice	Where the money left the building, case by case, before a naira was spent on attracting more of it.

Every one of those is a growth conversation that started with arithmetic. That is the difference between this practice and an agency.

The six questions that would decide the brief.

- **Which service lines carry your margin.** Not which are busiest. Which ones you would want more of if you could choose.
- **Where is the spare capacity, by site.** Lekki, Surulere and Maitama will not have the same answer, and the answer decides where a campaign points.
- **What do you want to be true in twelve months.** More elective volume, a fuller theatre list, a referral base, a payer contract, or a site in a fourth city. These need different work.
- **Who refers to you, and who has stopped.** The second half of that question is usually the more valuable one and almost nobody tracks it.
- **What happens to an enquiry.** Who answers, how fast, on which channel, and how many of them you could name a month later.
- **Is the six zone plan a capital plan or an ambition.** Both are legitimate answers. They imply completely different brand decisions now.

There is a longer written version of this, structured so most of it can be answered from memory. We will send it after the call if it is useful.

If this is what you need, this is what it looks like.

Shapes, not a quotation. A quotation needs the answers to the previous slide, and we would rather show you a build up than a round number.

If what you need is	Then expect	Shape
To know what is actually wrong before you spend anything	A marketing and growth diagnostic across the three centres, ending in a written read and a priority order	3 to 4 weeks
To settle who the group is, and who the sub brands are	A brand foundation engagement, with the architecture decision taken at the end of week two rather than the end of the project	4 to 6 weeks
To stop losing the enquiries you already generate	A digital front door build across the three centres, with the measurement installed and tested	6 to 8 weeks
To grow one service line that has capacity today	A service line launch: positioning, creative, campaign, referrer communications and a measurement frame	8 weeks
To package and sell the inbound surgical product	A destination product build: pathway, pricing, coordinator, complications promise and referrer relationships abroad	10 to 12 weeks
Ongoing delivery once a direction is set	A retainer, always on or growth, with a tightly defined scope and a change mechanism in the contract	Monthly

We price from resourced days at published grades. Media and third party production sit outside the fee and pass through at cost.

What we would like to happen next.

- **Today.** You talk and we ask. We will not pitch for the rest of the hour.
- **This week.** We send the written discovery set, and a two page note of what we heard, so you can check that we understood it.
- **Within a fortnight.** A working session with whoever owns the numbers, and a proposal against one shape rather than six.

> *We have run hospitals, so we build marketing around how a hospital actually earns.*

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